

Project Narrative

CDC-RFA-JG-26-0054 — Strengthening global health security through local partnerships in Uganda
FAIRBANKS MEDICAL CENTRE LIMITED (FairBanks Medical Centre) | Your health, our mission.

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1. Background and approach

1.1 Shared win

CDC's America First Global Health Strategy asks partners to help make America safer, stronger, and more prosperous by containing threats at source, building resilient systems, and using data for real response. Uganda's NAPHS II and the 2023 JEE ask for the same practical things on the ground: better surveillance links, ready workers, and emergency systems that reach districts and communities.

FairBanks sits where those two needs meet. We already walk the streets and homes of peri-urban Kampala. We already see patients at FairBanks Medical Centre. We already collect community data through CHWs/VHTs and a working FCHIP MVP. What CDC and MoH still need is a local partner that can turn that daily work into timely, government-usable signals — without inventing a second national system.

If funded, CDC gets a Uganda partner with skin in the game. MoH gets last-mile feed into NISS-aligned pathways and practised 7-1-7 loops. Communities get earlier alerts and clearer referrals. FairBanks gets to prove its tools serve the public system.

1.2 Problem

Uganda sits in a region where endemic, new, and returning infectious diseases keep causing outbreaks. Fast urban growth, crowded peri-urban settlements, and high border and travel movement raise the chance that a local cluster becomes a national — and cross-border — threat. Americans and Ugandans are safer when outbreaks are found and contained early, close to where people live.

In Kampala peri-urban communities where FairBanks already works — Bukoto, Kyebando, Kisaasi, Kamwokya, Kikaaya and nearby areas — families often reach care late. CHWs and VHTs visit homes and schools, but much of what they see still sits in paper books or separate tools. Clinic records, community reports, lab results, and weather or place signals rarely join one picture.

Uganda's 2023 Joint External Evaluation showed real progress on emergency response, labs, surveillance, and workforce — and still named clear gaps: weak information sharing across sectors; limited internet access for surveillance data; incomplete surge plans and staffing at subnational level; uneven use of data to forecast risk; and the need for stronger links from communities and facilities into national emergency and surveillance structures, including work toward a functional National Integrated Surveillance System (NISS).

FairBanks proposes to close the last-mile gap under MoH leadership: help community and clinic signals reach MoH systems faster, train the people who collect those signals, and practise response with district structures so 7-1-7 style timing becomes real on the ground.

1.3 Goal, theory of action, and outcomes

Goal: Strengthen Uganda's prevention, detection, and response capacities for priority human and zoonotic threats by linking FairBanks Community Reach, FairBanks Medical Centre, and FCHIP tools into Ministry of Health-led surveillance and emergency pathways — so outbreaks are found earlier and contained closer to source.

Theory of action: Train and equip frontline workers -> capture structured community and facility signals -> share into NISS-aligned / MoH channels -> trigger district and regional response -> review timing against 7-1-7 -> improve and hand tools to government use.

Asterisked outcomes we will substantially contribute to:

- Strategy 1: Improved health systems resilience for emergencies linked to districts and communities.
- Strategy 2: Increased capacity, coordination, and response readiness of the public health workforce.
- Strategy 4: Resilient integrated surveillance infrastructure capable of real-time detection and response; shorter detection and response time guided by 7-1-7 style benchmarks.
- Strategy 6: Better coordination between public health systems, clinical care, and communities; culturally responsive frontline workforce supporting public health operations.

For Strategies 3 (national laboratory systems) and 5 (border health), FairBanks will support MoH and specialised partners with referral pathways, specimen logistics coordination, private-facility incident reporting support where asked, and community risk communication near travel corridors — without claiming sole national lead.

Geographic start: Kampala Capital City Authority catchments linked to FairBanks Community Reach, with Year 1-2 expansion to selected neighbouring districts agreed with MoH/CDC.

1.4 Year 1 Component 1 activities by strategy

We build on IDSR/eIDSR, community-based surveillance, CHW/VHT structures, PHEOC incident management, 7-1-7 timing, One Health awareness, and RCCE. Tools will work offline first and respect Uganda data-protection and consent norms.

Strategy 1 — Health emergency management

- Map FairBanks catchments to district/regional PHEOC and agree notification SOPs.
- Run table-top and field drills with CHWs, facility staff, and district teams using 7-1-7 timing.
- Practise incident management for small events from community alert to district activation.
- Support district-level response coordination for events in FairBanks catchments.
- Document after-action reviews and corrective actions.

Strategy 2 — Human resources / workforce development

- Train and coach CHWs/VHTs and selected facility staff on community-based surveillance, event notification, and safe specimen/referral pathways.
- Support One Health-aware briefings with local animal/environmental focal points where MoH/districts arrange joint sessions.
- Build a surge roster of FairBanks-linked workers for community investigation and RCCE.
- Align training with MoH competency expectations; share materials with MoH and CDC.

Strategy 3 — National laboratory systems (support role)

- Strengthen sample referral and transport links from FairBanks Medical Centre and outreach points into MoH-approved laboratory pathways.
- Help MoH track private-facility coverage and routine incident reporting for priority outbreak diseases in our catchments.
- Support biosafety basics at facility sample handling points (SOPs, PPE, documentation).

Strategy 4 — Surveillance

- Deploy and validate FCHIP mobile capture for CHW/VHT and facility sentinel signals.
- Configure exports and dashboards that contribute to NISS/MoH pathways under MoH rules (not a parallel silo).
- Train staff on data quality, analysis, and simple visualisation for catchment and district review.
- Use GIS and approved climate/weather feeds where they help early warning, with clear limits on model claims.

Strategy 5 — Border health security (support role)

- Coordinate with MoH/border health partners on RCCE and community surveillance near high-mobility corridors serving Kampala catchments.
- Share community mobility and event signals that support POPCAB-style risk pictures when districts request them.
- Do not claim to run national Points of Entry programmes.

Strategy 6 — Public health programmes and service delivery links

- Strengthen community-facility linkages for priority diseases (GHS, HIV, TB, malaria, cholera, Ebola, Marburg, mpox, immunisation) during routine and outbreak periods.
- Support public health campaigns with community mobilisation and data feedback — not award-funded routine clinical care.
- Write SOPs for data use in service and public health decisions; train teams to use them.
- Provide MoH, CDC, and partners with copies of or access to tools, training materials, and systems developed under the award.

1.5 Complementing the national programme and timelines

This proposal continues the direction of prior CDC GHS investment in Uganda (including work under CDC-RFA-GH20-2124 as described in the NOFO). FairBanks adds the missing community last mile through the Community Reach cascade and FCHIP on the Data & Feedback loop, configured to feed government systems.

Quarter	Year 1 milestones
Q1	Staffing; MoH/district kick-off; SOP drafts; CHW roster; FCHIP form pack; EPMP/DMP drafts started
Q2	Train first cohorts; routine community event reporting; first joint drill; sample referral pathway live
Q3	Expand sites; mid-year data quality review; second drill; MoH data-sharing test exports
Q4	Annual performance package; Year 2 work plan; tool handoff package; surge roster exercise

2. Evaluation and performance measurement plan (EPMP)

About 5-10% of project funds will support monitoring, reporting, and evaluation. Final indicators will be agreed with CDC after award from the DGHP partner-level list.

2.1 Priority indicator areas

- Surveillance / community mitigation: staff trained in investigation/contact support; tailored risk mitigation strategies; RCCE knowledge gains.
- Emergency operations: trainings on RRT/RCCE/PHEM skills; SOPs for notification/IMS/RCCE; timely notification toward district activation (7-1-7 style).
- Laboratory (support): sample referral completeness and turnaround from FairBanks-linked sites.
- IPC: IPC focal person and guideline-based improvements at FairBanks Medical Centre and linked sites.

2.2 Project-specific measures (Year 1 plan)

Measure	Baseline plan	Year 1 target plan
Median days community signal to district notification	Establish Q1-Q2	Move toward 7-1-7; publish quarterly trend
CHWs/VHTs with complete weekly reports	Roster at kick-off	At least 80% of active roster by Q4
Priority referrals with documented outcome	Baseline audit Q1	At least 70% documented by Q4
Successful MoH/NISS-aligned data package tests	0	At least 2 by Q4
After-action reviews after drills/events	0	At least 2 by Q4

2.3 Methods, reporting, DMP summary

- Collect: mobile forms (offline), facility registers, training pre/post tests, drill logs, sample referral logs, dashboard extracts.
- Frequency: weekly ops checks; monthly internal review; quarterly packs to CDC; Annual Performance Report.
- Quality: form validation; supervisor spot checks; quarterly data-quality audits.
- Use: monthly improvement meetings; share with district/MoH; adjust SOPs and training.
- Evaluation: Year 2 process evaluation; optional Year 4 outcome evaluation if funds allow.
- DMP: role-based access; Uganda Data Protection and Privacy Act; minimise PII; de-identify analytics; detailed DMP within six months of award.

3. Organizational capacity and collaborations

FAIRBANKS MEDICAL CENTRE LIMITED (Company No. 80020003843337; TIN 1053370026; NSSF NS043295) is a Uganda company with principal operations in Kampala (Plot 1423 and 1425 Tirupati Road, Fairbanks Medical Centre, Kampala Central Division, Kololo IV, Lugogo). We run: (1) FairBanks Medical Centre — outpatient care, diagnostics, pharmacy, referrals, and related services; (2) FairBanks Community Reach — CHWs/VHTs, outreach, MCH support, GeriCare, school health, CHIS, and livelihood pathways; (3) FCHIP — working MVP for mobile capture, sync, dashboards, GIS, and secure data APIs.

Project Director: Racheal Nabukeera, Managing Director and Co-founder — 15+ years in Uganda private healthcare leadership and HR; MA Social Sector Planning and Management (Makerere); PhD in Management in progress; Uganda Healthcare Federation links. Contact: info@fairbanksmedicalcentre.org, +256772849258. Website: <https://www.fairbanksmedicalcentre.org/>.

Honest limit: we have not previously managed a multi-million-dollar U.S. federal cooperative agreement. We will strengthen segregated ledgers, procurement, timesheets, and audits for this award. We ask CDC to judge us on local presence, technical fit for community surveillance, documented company standing, and willingness to work under MoH leadership.

Collaborations (if funded):

- Ministry of Health (surveillance, community health, emergency operations, digital health).
- District / KCCA health teams covering our catchments.
- Other CDC-funded GHS, HIV/TB, immunisation, and emerging infection partners.
- Community leaders, CHWs/VHTs, schools, and local CBOs in Community Reach.

Key personnel

Role	Person	Notes
Project Director / Authorized Official	Racheal Nabukeera	Overall accountability
GHS Programme Manager	To hire / CONFIRM	Day-to-day delivery
M&E; Lead	To hire / CONFIRM	EPMP, indicators, APR
Data / FCHIP Lead	To hire / CONFIRM	Mobile tools, NISS-aligned exports
CHW Supervisor	Existing + surge	Field quality and coaching
Clinical / IPC Focal	Existing Medical Centre role	Sample SOPs/IPC — award-allowed time only

4. Work plan snapshots

4.1 Component 1 — Core GHS (Year 1)

Strategy	Activity	Measure examples	By
1	District notification SOPs + 2 drills	Drill reports; time metrics	Q2, Q4
2	Train CHW/VHT cohorts (target CONFIRM e.g. 80)	# trained; pre/post scores	Q2-Q3
3	Sample referral pathway SOPs live	% complete referrals	Q2
4	FCHIP surveillance live + MoH export tests	Weekly reports; export tests	Q2-Q4
5	Corridor RCCE + signal sharing	# sessions; signals shared	Q3-Q4
6	Priority-disease community-facility linkage	Completed referral outcomes	Ongoing
M&E;	EPMP/DMP detailed; quarterly reviews	Reports submitted	Month 6+

4.2 Components 2-4 — Contingency (approved but unfunded until CDC activates)

- Component 2 (\$400,000 draft): surge roster; community investigation/contact support; RCCE; temporary dashboards; district IMS support.
- Component 3 (\$500,000 draft): expand surge staffing; multi-district mobilisation under MoH; sample referral volume logistics; extended RCCE; recovery support.
- Component 4 (\$300,000 draft): rapid form updates for new pathogens; sentinel intensification; partner lab referral; special training modules.
- Component 5 (humanitarian emergency): contingency plan in this narrative for NOFO completeness. SF-424A has only four activity rows, so Year 1 federal dollars are on Components 1-4 totaling \$3,300,000. Component 5 would activate with CDC emergency funding under the \$20M ceiling.

Closing statement

FairBanks is ready to serve as a documented local partner for CDC-RFA-JG-26-0054: closing Uganda's last-mile Global Health Security gap under MoH leadership so threats are detected and contained closer to source — safer for Uganda and for the United States.

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